



अखिल भारतीय आयुर्विज्ञान संस्थान, जोधपुर
ALL INDIA INSTITUTE OF MEDICAL SCIENCES, JODHPUR

Department of Radiology

C T Lab

ABHA NO	:	ABHA ADDRESS	:	37532033304733@abdm
CR No	:	989262502007791	Lab/Study No.	: 260319CTL0030 Coll./Study Date : 19-Mar-26 11:08
Patient Name	:	Kishan Lal Bang	Age/Sex	: 48 Yr /M Acceptance Date : 19-Mar-26 11:08
Sample Type/No	:	NA	Ward/OPD	: OPD Reporting Date : 24-Mar-26 14:45
Dept/Unit	:	Cardio Thoracic And Vascular Surgery		

CORONARY CT ANGIOGRAPHY

Clinical profile: CABG, LIMA to diagonal, LRA to OM, RSVG to PDA.

Grafts:

LIMA to Diagonal : Patent. Diagonal to anastomosis – faintly opacified

LRA to OM : Patent. Normally opacified distal to anastomosis.

RSVG to PDA: Patent. Normally opacified distal to anastomosis.

Native vessels:

Dominance: Right

LMCA: Normal origin and course. No significant stenosis.

LAD: Normal origin and course. Diffuse atherocalcific plaques and severely attenuated in calibre causing approx. 70-80% luminal narrowing.

LCx: Normal origin and course. Calcified plaque at the ostioproximal segment causing 30-40% stenosis. Graft distal to OM appears patent.

Diagonal: Calcified plaque seen causing moderate to severe stenosis. Distal to anastomosis is patent with faint opacification.

RCA: Normal origin and course. Moderate to severe disease with multiple eccentric calcified atherosclerotic plaques seen in proximal and distal segment causing approx. 70-80% luminal narrowing.

PDA: Normal origin and course, Calcified plaque seen causing moderate to severe stenosis. Distal to anastomosis patent.

PLV: Normal origin and course, No significant stenosis.

Cardiac valves: Normal.

Cardiac chambers: LA and LV appear normal. No pericardial effusion seen.

LVEF: 78% , LVEDV: 131 cc.

Lungs: *Post op changes seen in form of sternotomy defect. Rest of the lung parenchyma appears normal.*

No significant abnormality seen.

Impression:

LIMA to Diagonal : Patent. Diagonal to anastomosis – faintly opacified.

LRA to OM : Patent. Normally opacified distal to anastomosis.

RSVG to PDA: Patent. Normally opacified distal to anastomosis.

All reports need clinical correlation. Kindly discuss if necessary. No part of the report can be reproduced without written permission of the department.

Validated By: Dr. Vishal Gupta



अखिल भारतीय आयुर्विज्ञान संस्थान, जोधपुर
ALL INDIA INSTITUTE OF MEDICAL SCIENCES, JODHPUR

Department of Radiology

C T Lab

ABHA NO	:	ABHA ADDRESS	:	37532033304733@abdm
CR No	:	989262502007791	Lab/Study No.	: 260319CTL0030 Coll./Study Date : 19-Mar-26 11:08
Patient Name	:	Kishan Lal Bang	Age/Sex	: 48 Yr /M Acceptance Date : 19-Mar-26 11:08
Sample Type/No	:	NA	Ward/OPD	: OPD Reporting Date : 24-Mar-26 14:45
Dept/Unit	:	Cardio Thoracic And Vascular Surgery		

Reported by: Dr. Palash (Junior Resident) / Dr. Siddhi Chawla (Assistant Professor)

Comments:

***** END OF THE REPORT *****

All reports need clinical correlation. Kindly discuss if necessary. No part of the report can be reproduced without written permission of the department.

Validated By: Dr. Vishal Gupta